

Findings Brief: Identifying Counties with Overlapping Postpartum Medicaid Access Barriers

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Research question: Where may postpartum Medicaid populations face overlapping administrative, clinical, and contextual access barriers, and which counties does the full index surface beyond a screen focused only on Medicaid office and hospital-based obstetric care gaps?

Why This Matters

Postpartum Medicaid access depends on more than formal eligibility. After delivery, people may need to maintain coverage, complete renewals, navigate documentation, find local assistance, and connect to postpartum or maternity-related care. Counties with limited in-person Medicaid support, no hospital-based obstetric care, transportation barriers, digital access barriers, language access needs, disability-related support needs, and rural geography may create overlapping conditions that make coverage and care navigation harder.

A screen focused only on whether a county has both no Medicaid office and no hospital-based obstetric care captures the most visible infrastructure gaps, but it can miss counties where one infrastructure gap combines with substantial contextual burden. This brief asks what the full index adds beyond that narrower screen.

What the Index Found

The index did more than identify counties with the most obvious infrastructure gaps. A dual-gap infrastructure screen flags counties only when they have both no Medicaid office and no hospital-based obstetric care. The full index also identified counties where one infrastructure gap, or no direct infrastructure gap, overlapped with substantial contextual access burden.

The key added-value finding: the full index adds value by identifying high-burden counties where contextual barriers elevate concern beyond what infrastructure gaps alone would show.

Measure	Result
Counties classified as high or highest concern	665
Captured by dual-gap infrastructure screen	386 / 665, 58.0%
Additional high/highest concern counties surfaced by the full index	279 / 665, 42.0%

Of the 665 counties classified as high or highest concern, 386 had both infrastructure gaps and would be captured by a dual-gap screen. The full index identified 279 additional high/highest concern counties. These additional counties were not low-burden outliers: they had an average score of 6.46 and an average contextual support-burden score of 4.56. Most were nonmetro, and large shares had high poverty, high no-internet rates, high disability rates, or high no-vehicle household rates.

This means the added-value group is not an unstructured residual category. Most of these counties follow a clear screening pattern: one major infrastructure gap plus multiple access-support barriers. In practical terms, the index helps distinguish counties that may warrant review even when they do not have both infrastructure gaps at once.

Counties Flagged for Further Review

The clearest case-review group is the 11 counties with the maximum observed score of 10. These counties were flagged because they combine both direct infrastructure gaps with multiple contextual access-support barriers. All 11 had no Medicaid office, no hospital-based obstetric care, high poverty, high no-vehicle household rates, high no-internet subscription rates, high disability rates, and nonmetro status. They differed mainly in whether the additional point came from high limited-English rate or high female population ages 15-44 rate.

County	State	Why flagged for review
Barbour County	AL	Both infrastructure gaps plus high poverty, no-vehicle, no-internet, limited-English, disability, and nonmetro flags.
Perry County	AL	Both infrastructure gaps plus high poverty, no-vehicle, no-internet, disability, female ages 15-44, and nonmetro flags.
Sumter County	AL	Both infrastructure gaps plus high poverty, no-vehicle, no-internet, disability, female ages 15-44, and nonmetro flags.
Evangeline Parish	LA	Both infrastructure gaps plus high poverty, no-vehicle, no-internet, disability, female ages 15-44, and nonmetro flags.
Madison Parish	LA	Both infrastructure gaps plus high poverty, no-vehicle, no-internet, disability, female ages 15-44, and nonmetro flags.
Humphreys County	MS	Both infrastructure gaps plus high poverty, no-vehicle, no-internet, disability, female ages 15-44, and nonmetro flags.

County	State	Why flagged for review
Noxubee County	MS	Both infrastructure gaps plus high poverty, no-vehicle, no-internet, disability, female ages 15-44, and nonmetro flags.
Harding County	NM	Both infrastructure gaps plus high poverty, no-vehicle, no-internet, limited-English, disability, and nonmetro flags.
Harmon County	OK	Both infrastructure gaps plus high poverty, no-vehicle, no-internet, limited-English, disability, and nonmetro flags.
Tillman County	OK	Both infrastructure gaps plus high poverty, no-vehicle, no-internet, limited-English, disability, and nonmetro flags.
Duval County	TX	Both infrastructure gaps plus high poverty, no-vehicle, no-internet, limited-English, disability, and nonmetro flags.

What the Index Adds

The index does not only ask whether a county has both major infrastructure gaps. It distinguishes administrative access, clinical maternity infrastructure, and access-support burden.

In other words, the index helps confirm where additional barriers matter within counties that would otherwise appear less urgent under a dual-gap-only screen.

Score Design

The Postpartum Medicaid Access Barrier Index is a transparent county-level screening score. The theoretical score range is 0-11, and observed county scores ranged from 0 to 10.

Component group	Fields	Points
Infrastructure gaps	No Medicaid office in county; no hospital-based obstetric care	2 points each
Contextual flags	Poverty; no vehicle; no internet; limited English; disability; female ages 15-44; nonmetro county status	1 point each

Applied Review Results

Applying the review sequence separates high-scoring counties into groups that would be reviewed for different reasons. This is the practical value of the index: it does not only rank counties, but also shows whether the score is driven by direct infrastructure gaps, one infrastructure gap plus high contextual burden, or contextual burden alone.

Review group	Counties	What this group means
Highest-concern counties	56	Counties scoring 9-10. This is the first review tier, including the 11 maximum-score counties listed above.
Dual infrastructure gaps	386	High/highest concern counties with both no Medicaid office and no hospital-based obstetric care.
Clinical-only gap plus high context	244	High/highest concern counties with no hospital-based obstetric care plus at least four contextual flags.
Administrative-only gap plus high context	21	High/highest concern counties with no Medicaid office plus at least four contextual flags.
Contextual-burden watchlist	14	High/highest concern counties with neither direct infrastructure gap but six contextual access-support flags.

The one-gap-plus-context and contextual-burden groups are the counties most likely to be left out by a narrower dual-gap-only screen. They are therefore important follow-up groups for stakeholder review, state comparison, and future linkage to Medicaid enrollment, renewal, churn, and postpartum coverage-continuity data.

Geographic Pattern

High scores were not evenly distributed across the country. The strongest state-level patterns were concentrated in the South, parts of the Southwest, and Alaska. Mississippi had the highest average county score and the highest share of counties in the high/highest concern categories. Louisiana had the largest number of highest-concern counties. Texas had the largest number of high/highest concern counties overall.

State pattern	Result	Interpretation for review
Highest average county scores	Mississippi 6.13; Louisiana 6.02; Alabama 5.67; New Mexico 5.52; Alaska 5.50; Oklahoma 5.29	These states show broad county-level access burden, not only isolated high-score counties.

State pattern	Result	Interpretation for review
Largest number of high/highest concern counties	Texas 89; Mississippi 54; Georgia 40; Oklahoma 40; Alabama 37; Kentucky and Louisiana 36 each	These states create the largest review workload if counties are prioritized by high/highest concern status.
Highest share of high/highest concern counties	Mississippi 65.9%; Alaska 60.0%; Louisiana 56.2%; Alabama 55.2%; Oklahoma 51.9%; New Mexico 48.5%	These states show the highest concentration of county-level burden relative to their number of counties.
Highest-concern county counts	Louisiana 12; Alabama 9; Mississippi 9; Texas 9; Oklahoma 6; New Mexico 4	These states should be examined first when the review starts with the most severe score category.

This geography suggests two complementary review strategies. A national review should start with the 56 highest-concern counties because they have the most overlapping barriers. A state-focused review should prioritize Mississippi, Louisiana, Alabama, Oklahoma, New Mexico, Alaska, and Texas because they combine high average scores, high shares of high/highest counties, or large counts of counties needing review.

Bottom Line

The most obvious infrastructure gaps are only part of the access-burden picture. The index provides a transparent way to identify counties that may be overlooked by simpler Medicaid access screens and to show where additional barriers make those counties worth closer review.